

UF HEALTH SHANDS CORE POLICY AND PROCEDURE

POLICY NUMBER: CP02.013

CATEGORY: Patient Care

REVIEW RESPONSIBILITY/DOCUMENT OWNER: Medical Director of Palliative Care

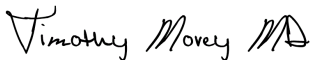
LAST REVIEWED: 9/16/25

TITLE: Withholding or Withdrawing Life-Prolonging Treatments or Measures

POLICY: At some point the patient's condition may be such that it is unreasonable to expect recovery, and some treatments may serve only to prolong the dying process. In this situation, the patient, or the patient's substitute decision-maker, has a right to request and receive the withholding or withdrawing of Life-Prolonging Treatments or Measures. It is UF Health Shands' policy to honor such requests.

PURPOSE: To clarify the legal requirements and establish policy and procedures for withholding or withdrawing life prolonging treatments or measures from patients.

APPROVED:

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DEFINITIONS:

- A. Advance Directive – A witnessed written document or oral statement in which instructions are given by a patient or in which the patient's desires are expressed concerning any aspect of the patient's health care. Examples include, but are not limited to, the Designation of a Health Care Surrogate, a Living Will, Anatomical Gift, or Health Care Durable Power of Attorney.
- B. Patient's Attending Physician – Means the primary physician having responsibility for the treatment and care of the patient.
- C. End Stage Condition – An irreversible condition that is caused by injury, disease, or illness which has resulted in progressively severe and permanent deterioration, and for which, to a reasonable degree of medical probability, treatment would be ineffective.
- D. Surrogate – Any competent adult expressly designated in writing or orally by a patient, during a time when the patient is capacitated, to make health care decisions on the patient's behalf during the patient's incapacity.
- E. Incapacity – Means the patient is physically or mentally unable to communicate a willful or knowing health care decision. A patient is presumed to have capacity unless there is written physician (M.D. or D.O.) documentation to the contrary.
- F. Life-Prolonging Treatments or Measures – Any medical procedure, treatment, or intervention, including artificial nutrition and hydration, which sustains, restores, or supplants a spontaneous vital function. This term does not include the administration of medication or performance of medical procedure when such medication or procedure is deemed necessary to provide comfort care or to alleviate pain.
- G. No Reasonable Probability of Recovering Capacity – The patient, in the medical judgment of the patient's Attending Physician and one other licensed physician who has examined the patient, has No Reasonable Probability of Regaining the Capacity to make his or her own decision regarding the withdrawal or withholding of Life-Prolonging Treatment or Measures. A "reasonable" standard implies that other reasonably prudent physicians in the community of comparable skill and expertise would come to a similar conclusion.
- H. Persistent Vegetative State – A permanent and irreversible condition of unconsciousness in which there is:
 - 1. The absence of voluntary action or cognitive behavior of any kind; and
 - 2. An inability to communicate or interact purposefully with the environment.
- I. Proxy – A competent adult who has not been expressly designated by a patient to make health care decisions for him/her in the event of incapacity, but who is chosen by the hospital in accordance with CP02.010 – Consent for Treatment and has the authority to make health care decisions for the incapacitated patient.

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- J. Terminal Condition – Condition caused by injury, disease, or illness from which there is no reasonable medical probability of recovery and which, without treatment, can be expected to cause death.
- K. Viability – That state of fetal development when the life of the unborn child may, with a reasonable degree of medical probability, be continued indefinitely outside the uterus.

CORE PROCEDURE:

I. Procedure for Capacitated Adults

- A. Life-Prolonging Treatments or Measures may be withheld or withdrawn from a capacitated adult upon the request of the patient to his/her Attending Physician.
- B. Prior to acting upon the patient's request, the Patient's Attending Physician will discuss his/her plan of care, the patient's condition, and the patient's request with the patient. Such discussion will be documented in the medical record.
- C. An order for withholding or withdrawing Life-Prolonging Treatments or Measures or other order as the circumstances dictate shall be entered in the medical record by the Patient's Attending Physician, or in the absence of the attending physician, by the fellow or senior resident, at the direction of the attending physician.

II. Procedure for Incapacitated or Incompetent Adults

- A. Life-Prolonging Treatments or Measures may be withheld or withdrawn from an incapacitated or incompetent adult patient in a Persistent Vegetative State or with a Terminal or End Stage Condition:
 - 1. if there is an Advance Directive addressing the withholding or withdrawing of life prolonging treatments or measures, in accordance with the directions and conditions of his/her Advance Directive, even if the patient has not designated a Surrogate to execute his or her wishes; or
 - 2. if there is no Advance Directive addressing the withholding or withdrawing of life prolonging treatments or measures, but there is a Surrogate, in accordance with the request of the patient's Surrogate, unless such person's power to make withholding and withdrawal decisions has been limited by the designation or by a court order; or
 - 3. if there is no Advance Directive addressing the withholding or withdrawing of life prolonging treatments or measures and no Surrogate has been designated, in accordance with the request of the patient's Proxy, who shall be chosen as outlined in CP02.010, "Consent for Treatment."
- B. The existence of an Advance Directive addressing the withholding or withdrawing of life prolonging treatments or measures or Surrogate designation should be documented in the designated area of the medical record.
- C. Prior to acting upon the Advance Directive or request of the patient's Surrogate or Proxy, the following procedures must be followed:

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1. The Patient's Attending Physician and one other attending physician shall each evaluate the patient and document in the medical record his/her opinion that the patient is in a Persistent Vegetative State or is suffering from a Terminal or End Stage Condition and that the patient has No Reasonable Probability of Recovering Capacity.
 2. If there is no Advance Directive addressing withholding or withdrawing of life prolonging treatments or measures, or if there is an Advance Directive in which the patient has designated a Surrogate to carry out the patient's wishes, the Patient's Attending Physician shall consult with the patient's Surrogate. The Patient's Attending Physician shall document the discussion with the patient's Surrogate and his/her consent to *Withholding or Withdrawing Life-Prolonging Treatment or Measures* in the progress notes.
- D. If a Surrogate has not been designated, consultation shall be with the patient's Proxy (See CP02.010 for order of priority for Proxy). Such consultation should be witnessed by at least one individual. The Patient's Attending Physician shall document the discussion with the patient's Surrogate/ Proxy, and document the patient's Surrogate's/ Proxy's consent to *Withholding or Withdrawing Life-Prolonging Treatment or Measures* in the progress notes.
- E. An order for withholding or withdrawing Life-Prolonging Treatments or Measures or other order as circumstances dictate shall be made in the medical record by the Patient's Attending Physician. If the Patient's Attending Physician is not physically available to enter these orders in the Medical Record, the orders may be entered by another Attending Physician, fellow or senior resident at the direction of the Patient's Attending Physician.
- F. The Patient's Attending Physician shall periodically examine the patient to verify the patient's condition. Should the patient's condition improve or circumstances change so that the patient is no longer in a Persistent Vegetative State or no longer suffers from a Terminal or End Stage Condition, the order for withholding or withdrawal of Life-Prolonging Treatments or Measures shall be rescinded.
- G. Health care Surrogates and Proxies who make treatment decisions for patients must make the decisions that they believe the patient would have made, if able, under the circumstances. If there is no indication of what the patient would have chosen, the decision should be based on the patient's best interest.

III. Procedure for Minors

- A. Life-Prolonging Treatments or Measures may be withheld or withdrawn from a minor patient in a Persistent Vegetative State or with a Terminal or End Stage Condition in accordance with the request of the patient's parent(s) or court-appointed guardian (unless such guardian's power to make withholding and withdrawal decisions has been limited by a court order).
- B. Prior to acting upon the request of the patient's parent(s) or court-appointed guardian, the following procedures must be followed:
 1. The Patient's Attending Physician and one other attending physician shall each evaluate the patient and document in the medical record his/her opinion that the patient is in a Persistent Vegetative State or is suffering from a Terminal or End Stage Condition.

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2. The Patient's Attending Physician shall consult with the patient's parent(s) or court-appointed guardian. Such consultation should be witnessed by at least one individual. If another individual is not available to witness such consultation, the Attending Physician may consult with the patient's parent(s) or court-appointed guardian without a witness. In either case, such consultation shall be documented by the Attending Physician in the patient's medical record.
- C. An order for withholding or withdrawing Life-Prolonging Treatments or Measures or other order as circumstances dictate shall be made in the medical record by the Patient's Attending Physician. If the Patient's Attending Physician is not physically available to enter these orders in the Medical Record, the orders may be entered by another Attending Physician, fellow or senior resident at the direction of the Patient's Attending Physician.
- D. The Patient's Attending Physician shall periodically examine the patient to verify the patient's condition. Should the patient's condition improve or circumstances change so that the patient is no longer in a Persistent Vegetative State or no longer suffers from a Terminal or End Stage Condition, the order for withholding or withdrawal of Life-Prolonging Treatments or Measures shall be withdrawn.

IV. Social Worker Proxy

A Social Worker Proxy should be appointed as a Proxy for an incapacitated patient having no family or friends. See CP02.098 – Appointment of Social Work Proxy for this process.

V. Refusal by Attending Physician to Honor an Advance Directive or Surrogate or Proxy Decision

Any Attending Physician who desires not to honor an Advance Directive or who refuses to comply with a treatment decision made by the patient's Surrogate or Proxy shall make a reasonable effort to transfer the patient to another attending physician or facility who will comply. Such transfer must occur within seven days of the determination that there is an inability to resolve the disagreement.

VI. Problem Resolution

If the Patient's Attending Physician disagrees with a patient, or patient's decision-maker, or when appropriate action is unclear, the following resources are available to the Attending Physician, patient, patient's family, and patient's decision-maker:

- A. Consultation with another attending physician who is acceptable to the patient, guardian, or appropriate decision maker; to include the option for consultation with the Palliative Care team.
- B. Referral to Pastoral Services or Patient and Family Resources;
- C. Meeting with an ethics advisory committee or appropriate sub-committee, which may include the patient, legal guardian, or appropriate decision-maker;
- D. Consultation with Legal Services; and
- E. Consultation with the facility's Chief of Staff or Chair of the Operations Committee.

VII. Pregnant Patients

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Unless such authority has been expressly granted to a Surrogate by the patient, or unless the Surrogate or Proxy has obtained court approval, a Surrogate or Proxy may not consent to withholding or withdrawing of Life-Prolonging Treatments or Measures from a pregnant patient prior to Viability. Requests for withholding or withdrawing of life-prolonging treatment or measures on a pregnant woman after Viability should be discussed with Legal Services.

VIII. Do Not Resuscitate Orders (DNR)

For the entry of a Do Not Resuscitate order, see CP02.012 – “Do Not Resuscitate” Orders.

- IX. Supportive guidelines for this policy are available as an additional resource for the healthcare team. See Publication Services Form [PS145313](#).

ASSOCIATED POLICIES:

CP02.010 – Consent for Treatment
CP02.012 – "Do Not Resuscitate" Orders
CP02.098 – Appointment of Social Work Proxy

KEY WORDS: Withdrawal, Living Will, End of Life, Terminal, End Stage